APEXCARE INDIA EMPLOYEE HEALTH BENEFIT PROGRAMME (AIEHBP)
CASHLESS, NETWORK AND PREAUTHORIZATION GUIDE

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Primary audience: Members, network hospitals, TPA and utilization teams

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Document-control rule: cite the document code, section, clause, page, and version in every decision.
Interpretation rule: the signed policy schedule and endorsements override this demonstration text.

1. NETWORK PRINCIPLES

Cashless service is a payment arrangement, not a separate coverage guarantee.

NET-1.1 Network status

A hospital is network only when contracted and active for the date of admission. Directory information must be verified at request time.

NET-1.2 Member choice

Members may use non-network providers subject to reimbursement terms, emergency rules and any schedule restrictions.

NET-1.3 No balance-billing promise

The member remains responsible for non-covered items, co-payment, deductible, excess over limits and provider charges not accepted under the network tariff.

2. PLANNED ADMISSION WORKFLOW

Planned treatment should be assessed before admission.

NET-2.1 Submission timing

The hospital should submit a complete preauthorization request at least forty-eight hours before planned admission.

NET-2.2 Clinical content

The request must identify member, diagnosis, symptoms, investigations, proposed procedure, estimated charges, room class and expected stay.

NET-2.3 Possible outcomes

The request may be authorized, partially authorized, queried, declined for cashless service, or referred for medical review. Declined cashless service is not automatically a final claim rejection.

NET-2.4 Change in treatment

Material change in diagnosis, procedure, implant, room or estimate requires enhancement or revised authorization.

3. EMERGENCY ADMISSION WORKFLOW

Emergency care must not be delayed solely for administrative authorization.

NET-3.1 Notification

Hospital or member should notify the TPA within twenty-four hours or as soon as practicable.

NET-3.2 Stabilization

Necessary stabilization proceeds according to clinical judgement. Authorization may be retrospective when immediate records were unavailable.

NET-3.3 Emergency evidence

Emergency notes should describe onset, vital findings, provisional diagnosis, immediate treatment and reason admission could not await planned authorization.

4. AUTHORIZATION CONTROLS

Authorization must be traceable and limited to evidence reviewed.

NET-4.1 Authorization identifier

Every decision receives a unique identifier, timestamp, reviewer and versioned evidence list.

NET-4.2 Scope

Authorization identifies approved diagnosis or procedure, room eligibility, estimated amount, conditions and exclusions reserved for final assessment.

NET-4.3 Enhancement

An enhancement request states the additional amount and clinical reason. Earlier approval is not erased when an enhancement is declined.

NET-4.4 Validity

Authorization is valid only for the named member, provider, admission episode and validity period.

5. DISCHARGE AND FINAL SETTLEMENT

The hospital submits final evidence before discharge settlement.

NET-5.1 Final packet

Final bill, breakup, discharge summary, procedure notes, pharmacy and diagnostic details, implant evidence and member acknowledgment are required as applicable.

NET-5.2 Reconciliation

Compare estimated and final diagnosis, procedure, stay, room and cost. Explain every material variance.

NET-5.3 Member collection

The provider may collect documented non-payable amounts. Undifferentiated deposits or unexplained deductions must be escalated.

NET-5.4 Post-settlement audit

Payment may be audited, but recovery requires evidence, authority and due process.

6. NON-NETWORK AND REIMBURSEMENT CONVERSION

Cashless unavailability must not be presented as proof that treatment is excluded.

NET-6.1 Conversion
A member may submit reimbursement documents when cashless service is unavailable or declined, subject to policy terms.

NET-6.2 Emergency non-network care
Emergency non-network hospitalization is assessed on necessity and reasonable charges; failure to reach a network hospital must be documented where relevant.

NET-6.3 Provider not recognized
Claims from an unlicensed, excluded or unverifiable provider require compliance review and may be inadmissible under EXC-6.

7. COMMUNICATION TEMPLATES

Communications must separate administrative status from final coverage.

NET-7.1 Query language

State the exact information required, its purpose and the response channel. Avoid language implying rejection before review.

NET-7.2 Partial authorization

Identify approved amount, reserved items, member cost share and possibility of final adjustment.

NET-7.3 Cashless decline

Explain whether the issue is provider arrangement, insufficient evidence, policy exclusion or non-eligibility, and state whether reimbursement remains available.

8. SERVICE MONITORING

Operational metrics must not reward unsafe automatic approval or denial.

NET-8.1 Metrics

Monitor response time, query rate, avoidable resubmission, member complaints, overturned decisions, leakage and provider variance.

NET-8.2 Quality sampling

Sample authorized, declined and enhanced cases for clause accuracy, clinical rationale and communication quality.

NET-8.3 System fallback

During outage, record requests through approved continuity channels and preserve original receipt time.